

Royce Pharma Manufacturing Sdn. Bhd.
Chloramine Syrup 4mg/5ml

Section D3 – Package Insert (front)

Storage Condition:

Keep container tightly closed. Store in a dry place (below 30°C).
Protect from light.

Shelf Life:
3 years

Pack Size:

Plastic bottle containing 60ml, 90ml, 100ml, and 120ml.

Product Registration Number:
MA19861150AZ

Further information can be obtained from your doctor or
pharmacist.



Product Holder / Manufactured by:
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RPI 0010-3
101016

ROYCE®

CHLORAMINE Syrup
4mg/5ml

(Chlorpheniramine Maleate-Antihistamine)

Presentation:

A yellow coloured, clear syrup with pineapple flavour.

Contents:

Each 5ml contains:
Chlorpheniramine Maleate 4mg

Preservatives:

Sodium Benzoate 0.1%w/v
Methylparaben 0.1%w/v
Propylparaben 0.01%w/v

Pharmacological Information:

Antihistamines competitively antagonize most of the smooth muscle stimulating actions of histamines on the H₁-receptors of the GI tract, large blood vessels, and bronchial muscle. The drugs are only feebly antagonistic to bronchospasm induced by antigen-antibody reactions. Antihistamines also effectively antagonize the action of histamine that results in increased capillary permeability and the formation of edema, H₁-receptors antagonists also suppress flare and itch which accompany the endogenous release of histamine. Antihistamines appear to act by blocking H₁-receptors site, thereby preventing the action of histamine on the cell; they prevent the release of histamine. Antihistamine do not block the stimulating effect of histamine on gastric acid secretion, which is mediated by H₂-receptors of the parietal cells. The basic ethylamine group is common to antihistamines and to anticholinergics, ganglionic and adrenergic blocking agents, local anesthetics and antispasmodics; antihistamines also may be expected to enhance the pressure action of norepinephrine. The antiemetic and anti-motion-sickness actions of some antihistamines appear to result, at least in part, from their central anticholinergic and CNS depressant properties. The effects of diphenhydramine on parkinsonism and drug-induced extrapyramidal reactions are also apparently related to its central anticholinergic effects.

Section D3 – Package Insert (back)

The drugs reportedly are well absorbed following oral or parental administration, but various salts may differ in activity and toxicity because of differences in solubility or absorption. The least soluble antihistamines are often the least toxic and may have a slow onset but prolonged duration of action. Following oral administration of antihistamines, symptomatic relief of allergic reactions usually begins within 15-30 minutes and is usually fully developed within 1 hour. The duration of action is variable but symptoms are usually relieved for 3-6 hours after oral administration of most antihistamine.

Some of the drugs have a longer duration of action. There may be some decrease in effectiveness with prolonged use of these drugs, although a substantial degree of tolerance to the antihistamines effects does not generally occur. However, tolerance to the sedative effects may occur. The distribution of most antihistamines have not been studied. Those compounds that have been studied show highest concentration in the lungs and lower concentrations in spleen, kidneys, brain, muscle and skin. Small amounts to the drugs appear to be distributed in the milk. The metabolic fate of most antihistamines has not been studied. The drugs appear to be extensively metabolized, chiefly in the liver and excreted on the urine as inactive metabolites within 24 hours. Negligible amounts of the drugs are excreted unchanged in the urine.

Indication:

Chloramine is indicated for symptomatic control of all allergic conditions responsive to antihistamines, including hay fever, nasal irritation and the watery discharge, urticaria and skin irritations.

Dosage and Administration:

For oral use only

Adult: 5ml, 3 or 4 times daily.

Children:

2-5 years: 2.5ml, 3 times daily.

6-12 years: 2.5ml-5ml, 3 or 4 times daily.

Contraindication:

Hypersensitivity to Chlorpheniramine or structurally related antihistamines.

Warning & Precautions:

This medicine may cause drowsiness. If so, patient should not drive or operate machinery.

Caution is required when there is concurrent usage of alcohol or other CNS depressants (see Drug Interaction)

Elderly patients - Dizziness, sedation and hypotension occur more frequently in patients over 60 years of age.

Special-risk patients - Use with caution in patients with narrow-angle glaucoma, stenosing peptic ulcer, pyloroduodenal obstruction, urinary bladder obstruction (owing to symptomatic prostatic hypertrophy or narrowing of the bladder neck, increased intraocular pressure, hyperthyroidism, cardiovascular disease, hypertension, or a history of bronchial asthma.

Interference with skin tests - Antihistamines may prevent or diminish otherwise positive reactions to allergens used in skin tests, discontinue Chlorpheniramine about 4 days before a skin test.

Patients receiving anticoagulants - Chlorpheniramine should not be used in combination with oral anticoagulants since it may reduce their effectiveness.

Carcinogenicity, mutagenicity - Long-term studies in rats fed with Chlorpheniramine have shown no evidence of carcinogenicity; no signs of mutagenicity due to Chlorpheniramine or its nitrosation metabolite have been seen in the Ames test.

Effect on fertility - No adverse effects on fertility have been detected in rats fed up to 67 times the human dose.

(a) Chloramine Syrup 4mg/5ml should not be used in pediatric patients less than 2 years of age.

(b) To be used with caution and doctor's/ pharmacist's advice in children 2 to 6 years of age.

Pregnancy & Lactation:

Pregnancy:

Unnecessary administration of drugs during the first trimester of pregnancy is undesirable.

Nursing mothers:

It is not known whether chlorpheniramine is excreted in human milk; however, low concentrations of other antihistamines have been detected in human milk. Because of the increased risk of antihistaminic side effects in infants, particularly in neonates, patients should not nurse while taking this drug.

Side Effects:

Cardiovascular - Hypotension, headache, palpitations, tachycardia and extrasystoles.

Hematological - Hemolytic anemia, hypoplastic anemia, thrombocytopenia and agranulocytosis.

Central nervous system - Drowsiness, sedation, dizziness, disturbed coordination, fatigue, confusion, restlessness, excitation, nervousness, tremor, irritability insomnia, euphoria, paresthesias, vertigo, tinnitus, acute labyrinthitis hysteria, neuritis and convulsions.

Ophthalmic - Blurred vision and diplopia.

Gastrointestinal - Epigastric distress, anorexia, nausea, vomiting, diarrhoea and constipation.

Genitourinary - Urinary frequency, difficult urination, urinary retention and early menses.

Respiratory - Thickening bronchial secretions, tightness in the chest and wheezing and nasal stuffiness.

Dermatological - Urticaria, drug rash and photosensitivity. Other - Anaphylactic shock, excessive perspiration, chills, dry mouth, nose and throat.

Interactions with Other Medicaments:

Alcohol, tranquilizers, barbiturates, sedative-hypnotics, and other CNS depressants. - Increase CNS depression.

MAO inhibitors - Increase anticholinergic effects.

Symptoms and Treatment of Overdose:

The estimated lethal dose of Chlorpheniramine is 25-50mg per kg body weight. Treatment should include gastric lavage if massive overdosage has been by the oral route. In the event of convulsions sedate with intramuscular paraldehyde. Severe respiratory depression may necessitate mechanical ventilation. Severe hypotension may require fluid replacement.